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GUIDELINE FOR BORDERLINE PRODUCTS

This guideline provides information on the process to be followed when the appropriate regulatory pathway for a specific product is unclear. It also outlines the criteria to be applied to determine whether the product is regulated in terms of the Medicines and Related Substances Act, 1965 (Act 101 of 1965). It should be noted that regulatory decisions relating to borderline products will be made on a case-by-case basis. Applicants are urged to comply with the administrative requirements when submitting applications relating to borderline products to facilitate efficient processing. The South African Health Products Regulatory Authority reserves the right to request additional information where necessary.

Document History

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Glossary

Abbreviation/ Term	Meaning
Borderline Product	A product for which the appropriate regulatory pathway as a medicine, medical device, cosmetic, foodstuff, disinfectant, or other consumer product is unclear
Cosmetic (<i>Foodstuffs, Cosmetics and Disinfectants Act, 1972 (Act 54 of 1972)</i>)	Any article, preparation or substance (except a medicine as defined in the Medicines and Related Substances Act, 1965 (Act 101 of 1965) intended to be rubbed, poured, sprinkled or sprayed on or otherwise applied to the human body, including the epidermis, hair, teeth, mucous membranes of the oral cavity, lips and external genital organs, for purposes of cleansing, perfuming, correcting body odours, conditioning, beautifying, protecting, promoting attractiveness or improving or altering the appearance, and includes any part or ingredient of any such article or substance (Note: Definition amended by Act 39 of 2007)
DES	Drug Eluding Stent
Disinfectant (<i>Foodstuffs, Cosmetics and Disinfectants Act, 1972 (Act 54 of 1972)</i>)	Any article or substance used or applied or intended to be used or applied as a germicide, preservative, or antiseptic, or as a deodorant or cleansing material which is not a cosmetic
Foodstuff (<i>Foodstuffs, Cosmetics and Disinfectants Act, 1972 (Act 54 of 1972)</i>)	Any article or substance (except a medicine as defined in the Medicines and Related Substances Act, 1965 (Act 101 of 1965) ordinarily eaten or drunk by a person or purporting to be suitable, or manufactured or sold, for human consumption, and includes any part or ingredient of any such article or substance, or any substance used or intended or destined to be used as a part or ingredient of any such article or substance
Health product	A medicine, medical device, or scheduled substance as regulated in terms of the Medicines and Related Substances Act, 1965 (Act 101 of 1965)
IVD	<i>In-vitro</i> Diagnostics
Medical device (<i>Medicines and Related Substances Act, 1965 (Act 101 of 1965)</i>)	Any instrument, apparatus, implement, machine, appliance, implant, reagent for <i>in vitro</i> use, software, material, or other similar or related article, including Group III and IV Hazardous Substances contemplated in the Hazardous Substances Act, 1973 (Act 15 of 1973) (a) intended by the manufacturer to be used, alone or in combination, for humans or animals, for one or more of the following: (i) diagnosis, prevention, monitoring, treatment, or alleviation of disease; (ii) diagnosis, monitoring, treatment, alleviation of, or compensation for an injury; (iii) investigation, replacement, modification, or support of the anatomy or of a physiological process; (iv) supporting or sustaining life;

Abbreviation/ Term	Meaning
	(v) control of conception; (vi) disinfection of medical devices; or (vii) providing information for medical or diagnostic purposes by means of <i>in vitro</i> examination of specimens derived from the human body; and which does not achieve its primary intended action by pharmacological, immunological, or metabolic means, in or on the human or animal body, but which may be assisted in its intended function by such
Medicine <i>(Medicines and Related Substances Act, 1965 (Act 101 of 1965))</i>	(a) any substance or mixture of substances used or purporting to be suitable for use or manufactured or sold for use in— (i) the diagnosis, treatment, mitigation, modification, or prevention of disease, abnormal physical or mental state, or the symptoms thereof in humans; or (ii) restoring, correcting, or modifying any somatic or psychic, or organic function in humans, and (b) includes any veterinary medicine
Scheduled Substance <i>(Medicines and Related Substances Act, 1965 (Act 101 of 1965))</i>	Any medicine or other substance prescribed by the Minister under section 22A of the Medicines and Related Substances Act, 1965 (Act 101 of 1965)
TAC	Technical Advisory Committee
WHO	World Health Organization

1. INTRODUCTION

Health products that aim to diagnose, treat, or prevent disease and injury include all medicines and medical devices (including *in vitro* diagnostic devices). Boundaries may sometimes be blurred, with products coming to market which are difficult to distinguish as a medicine, medical device, cosmetic, disinfectant, or foodstuff. A lack of clarity in such cases may lead to overlap or conflicting regulatory requirements. In some cases, the product may escape regulation with no legislation specific to that product being applied.

According to the World Health Organization (WHO), “borderline products are generally medical products for which it is unclear as to which legislation applies. Although such products may have some of the attributes of two or more categories of regulated products, they are not necessarily combination products.” The WHO advises that “to be predictable and transparent, the regulator should develop criteria and mechanisms for determining the appropriate regulatory regime for such products through a guideline. It should describe considerations and the process whereby an applicant may obtain an advisory opinion from the regulatory authority.”¹ Where necessary, the process should allow for consultation with subject matter experts, the manufacturer concerned, as well as with other regulators as needed. It may also consider determinations made by regulatory authorities in other jurisdictions.

1.1 Purpose

This guideline sets out the process to determine the appropriate regulatory pathway to be applied in the regulation of borderline products. This approach will help to provide applicants with routing to the relevant competent authority and avoid “regulatory orphaning” for products not clearly regulated under the Medicines and Related Substances Act, 1965 (Act 101 of 1965) (the Medicines Act), or the Foodstuffs, Disinfectants and Cosmetics Act, 1972 (Act 54 of 1972) (the Foodstuffs, Cosmetics & Disinfectants Act).

1.2 Scope

This document provides guidance on borderline products which could either be classified as medicines or medical devices under the Medicines Act, products classified as cosmetics, disinfectants or foodstuff under the Foodstuffs, Cosmetics and Disinfectants Act as well as other consumer products which are not regulated by these two pieces of legislation. The scope includes combination products which, for purposes of this guideline, may contain a scheduled substance, medicine, and/or a medical device. This guideline does not include products that are, for example, regulated under the Fertilizers, Farm Feeds, Agricultural Remedies

¹ WHO QAS/16.664, 2026.

and Stock Remedies Act, 1947 (Act 36 of 1947).

2. LEGAL PROVISIONS

According to Section 2A of the Medicines Act, the objects of SAHPRA are to monitor, evaluate, regulate, investigate, inspect, register, and control medicines, scheduled substances, clinical trials, medical devices, including in-vitro diagnostics (IVDs), and related matters in the public interest. The legal definitions of a *“medicine, medical device, and a scheduled substance”* are provided in Section 1 of the same Act, while Section 15 provides for the registration of medicines, medical devices, and IVDs.

If SAHPRA is satisfied that a medicine or medical device should be registered in terms of section 15(3)(a) of the Medicines Act, SAHPRA may register the medicine or medical device subject to conditions determined in terms of section 15(6). Before embarking on the process of application for registration of a product, a decision may, however, be needed as to whether the product in question does indeed fall within the ambit of the Medicines Act. This guideline has been developed to guide that decision-making process.

It must be noted, however, that products regulated under both the Medicines Act and the Foodstuffs, Cosmetics and Disinfectants Act may in addition to specific regulatory control under these Acts, also be regulated by aspects of other legislation including but not limited to:

- the National Health Act, 2003 (Act 61 of 2003);
- the Hazardous Substances Act, 1973 (Act 15 of 1973);
- the National Regulator for Compulsory Specification Act, 2008 (Act 4 of 2008); or
- the Legal Metrology Act, 2014 (Act 9 of 2014).

In addition, all products which fall under the definition of ‘goods’ as defined in the Consumer Protection Act 2008 (Act 68 of 2008) are also regulated by that Act.

3. HEALTH PRODUCTS

3.1 Medicine

A medicine is defined in the Medicine Act as:

“(a) any substance or mixture of substances used or purporting to be suitable for use or manufactured or sold for use in—

- (i) *the diagnosis, treatment, mitigation, modification, or prevention of disease, abnormal physical or mental state or the symptoms thereof in humans; or*
 - (ii) *restoring, correcting, or modifying any somatic or psychic, or organic function in humans, and*
- (b) *includes any veterinary medicine.”*

A substance or mixture of substances may be classified as a medicine based on what it is used for, or is claimed to be suitable for use for, as per the definition provided above. The action of a medicine may be achieved in, or on the body of a human or animal, by one or more of the following, or any other mechanism:

- Pharmacological - meaning an interaction typically at a molecular level between a substance or its metabolites and a constituent of the human body which results in the initiation, enhancement, reduction, or blockade of physiological functions or pathological processes;
- Immunological - meaning an action in or on the body that occurs through the stimulation and/or mobilisation of cells and/or products involved in a specific immune reaction; or
- Metabolic - meaning an action which involves an alteration of the normal chemical processes participating in, and available for, normal body function. Such an action could involve the substance or one or more of its metabolites stopping, starting or changing the rate, extent or nature of a biochemical process, whether physiological or pathological.

The fact that a product is itself metabolised does not, however, imply that it achieves its principal intended action by metabolic means.

3.2 Medical devices

A medical device is defined in the Medicines Act as:

“Any instrument, apparatus, implement, machine, appliance, implant, reagent for in vitro use, software, material, or other similar or related article, including Group III and IV Hazardous Substances contemplated in the Hazardous Substances Act, 1973 (Act 15 of 1973)—

- (a) *intended by the manufacturer to be used, alone or in combination, for humans or animals, for one or more of the following:*
 - (i) *diagnosis, prevention, monitoring, treatment, or alleviation of disease;*

- (ii) *diagnosis, monitoring, treatment, alleviation of, or compensation for an injury;*
 - (iii) *investigation, replacement, modification, or support of the anatomy or of a physiological process;*
 - (iv) *supporting or sustaining life;*
 - (v) *control of conception;*
 - (vi) *disinfection of medical devices; or*
 - (vii) *providing information for medical or diagnostic purposes by means of in vitro examination of specimens derived from the human body; and*
- (b) *which does not achieve its primary intended action by pharmacological, immunological or metabolic means, in or on the human or animal body, but which may be assisted in its intended function by such.*

Where a health product does not achieve its primary intended purpose by pharmacological, immunological, or metabolic means, the product is designated as a medical device, e.g. coronary stent. Such a product may, however, be assisted in that effect or purpose by pharmacological, immunological, or metabolic means, e.g., a coronary stent coated with a scheduled substance, referred to as a drug eluting stent (DES).

A product that is a “related article,” is included in the definition of medical device e.g. operating software in a medical device or a foot switch which is necessary to operate a medical device, is itself classified as a medical device.

3.3 Combination products

In the case of medicines and medical devices, a combination product means a product which may be comprised of two or more components regulated as a medicine and a medical device e.g. a syringe prefilled with a medicine. These components may be physically, chemically, or otherwise combined or mixed and presented as a single entity. A medical device may contain a medicine or a scheduled substance which acts on the body in a manner ancillary to the device. However, where such a substance(s) acts in a manner that is the primary intended mode of action, the product is regulated as a medicine rather than a medical device e.g. medicine provided in a metered dose inhaler to treat asthma. Refer to *SAHPGL Guideline for Classification of Medical Devices*.

4. NON-HEALTH PRODUCTS

4.1 Cosmetic

A cosmetic is defined in the Foodstuffs, Cosmetics and Disinfectants Act, 1972 as:

“Any article, preparation or substance (except a medicine as defined in the Medicines and Related Substances Act, 1965 (Act 101 of 1965)) intended to be rubbed, poured, sprinkled or sprayed on or otherwise applied to the human body, including the epidermis, hair, teeth, mucous membranes of the oral cavity, lips and external genital organs, for purposes of cleansing, perfuming, correcting body odours, conditioning, beautifying, protecting, promoting attractiveness or improving or altering the appearance, and includes any part or ingredient of any such article or substance;”

Cosmetics are applied to external parts of the human body with the intended purpose as included in the definition. There is a broad range of products at the cosmetic-medicine-medical device interface. A therapeutic claim may not be made for a cosmetic. In addition, consideration must be given to whether the product is being used, purporting to be suitable for use or manufactured or sold for use for the purposes described in the definition of a medicine or medical device. If such a claim is made and depending on the representation of its intended use and/or composition, the provisions of the Medicines Act may apply.

In cases where there is doubt, the appraisal should focus on whether the product is a medicine or a medical device. Thus, if the product falls within the definition of a medicine or a medical device and/or a therapeutic claim is made (e.g. reference made ‘to treat or prevent skin disease’), it must be assessed under the medicine or medical device framework. If neither a medicine nor a medical device framework is applicable it could, for example, be classified as a cosmetic.

4.2 Disinfectant

A disinfectant is defined in the Foodstuffs, Cosmetics and Disinfectants Act, 1972 as:

“Any article or substance used or applied or intended to be used or applied as a germicide, preservative or antiseptic, or as a deodorant or cleansing material which is not a cosmetic.”

A sanitising product may be classified as a medicine, medical device or disinfectant depending on the:

- (a) intended use and function of the product;
- (b) composition of the product;
- (c) surface to which it is applied e.g. the skin of a person or animal or an inanimate surface; and

- (d) the environment where the product is intended for use.

For example, a hand sanitiser for personal hygiene might be considered to be a cosmetic if marketed only for cleansing, but if claims are made that it is antibacterial, it may be classified as a disinfectant or medicine. If, the product's claims or use align with any human or veterinary therapeutic intent, it is a medicine rather than a disinfectant. Disinfectants used to clean medical instruments and inanimate surfaces in for example, hospitals, operating rooms, intensive care units and burn units will be controlled as medical devices under the Medicines Act.

4.3 Foodstuff

A foodstuff is defined in the Foodstuffs, Cosmetics and Disinfectants Act, 1972 as:

“Any article or substance (except a medicine as defined in the Medicines and Related Substances Act, 1965 (Act 101 of 1965)) ordinarily eaten or drunk by a person or purporting to be suitable, or manufactured or sold, for human consumption, and includes any part or ingredient of any such article or substance, or any substance used or intended or destined to be used as a part or ingredient of any such article or substance;

In cases where there is doubt, the first consideration must be whether or not the product falls within the definition of a medicine. Products marketed as foods or supplements should not make therapeutic claims as doing so will result in classification as a medicine.

4.4 Other consumer products

In addition, there are other products which **do not** meet the definition of either a medicine, medical device, foodstuff, cosmetic, or disinfectant and are intended for purposes or uses not covered by these product types. Such products may include, but are not limited to, exercise equipment, software applications and programmes, and information and communication technology. If, however, a product such as a fitness tracker has the functionality of a medical device (e.g. electrocardiogram (ECG) monitoring in watches) it should be regulated as a medical device.

5. BORDERLINE PRODUCTS

The process for classification of a borderline product should be initiated when there is a lack of clarity or dispute regarding the correct regulatory pathway to be followed for the classification of the product. In dealing with these situations, the first step is to decide whether or not the product is regulated under the Medicines Act. If it is determined that the product should be regulated by SAHPRA, a recommendation may be made regarding the appropriate regulatory pathway to be applied.

5.1 Criteria

In order to decide the regulatory pathway appropriate for a borderline product, the following factors will be considered:

- (a) The **legal definitions** of a medicine, medical device, foodstuff, cosmetic, and disinfectant as provided in the Medicines Act, and the Foodstuffs, Cosmetics and Disinfectants Act, **MUST** be considered first. In addition, it must be ascertained whether the product itself or a component of the product has been called up for registration in terms of section 14(2) of the Medicines Act. It is also important to check whether the product or a component thereof has been declared as undesirable in terms of section 23 of the Medicines Act in the case of medicines for human use, or section 36A in the case of veterinary medicines.
- (b) Consideration will also be given to the primary **intended purpose and/or use** of the product, including any **claim(s) made** for the product. Both explicit and implicit claims are considered when a product is classified. Explicit claims are stated clearly with no ambiguity regarding their meaning or intent. In contrast, implicit claims use wording which may suggest a therapeutic benefit in an indirect manner (for example -Product X is 'clinically proven').

Even if no therapeutic claim is made (either explicit or implicit), a product may be classified as a medicine or a medical device, if the intrinsic properties of the product are such that there is no other possible use. For example, paracetamol has no other use but as a medicine.

When considering the claim made for a product, it is important to note whether more than one property can contribute to the overall effect of the product when used for this indication. To make the necessary distinction, the applicant should, where appropriate do a risk assessment. Specifically, if it is determined that the greater risk is associated with the modification of any somatic, psychic, or organic function, then the safety, quality, and efficacy of the product would be more appropriately assessed under the medicine framework. The reverse will also apply.

- (c) The method by which the **primary mode of action** is achieved, e.g., through pharmacological, immunological, or metabolic means must be considered. The primary mode of action of the product means the most important therapeutic action that makes the greatest contribution to the overall intended therapeutic effect(s) of the product. For example, the primary mode of action of a metered dose inhaler containing a scheduled substance used to treat asthma, is the pharmacological action of the scheduled substance included in the product, and thus it is regulated as a medicine. In another example, however, the primary mode of action of a drug-eluting stent, is for the stent to keep the blood vessel open. The scheduled substance coated onto the stent has the secondary role of reducing the risk of blood clotting in the blood vessel. The product is thus regulated as a medical device.
- (d) The **composition of the product** - the presence of, for example, a scheduled substance, in a product that is making a therapeutic claim, would make the product unsuitable for designation as a cosmetic or foodstuff.
- (e) The **classification of similar products** on the market and/or in other regulatory jurisdictions may be considered by SAHPRA but will not be binding. It must be emphasised that classification is done on a case-by-case basis with that applied in other jurisdictions used for guidance only, with final decisions based on South African legislation and context.

The criteria above may not be given equal weight in support of a decision, as some factors may be more important to consider in one circumstance versus another. Not all criteria need to be applied if a decision can be reached after the examination of one or a few criteria. If the standard criteria do not lead to a clear decision, supplementary considerations may be considered. The final decision is dependent on the overall consideration. See decision tree in Annexure A

Once a product has been classified as a medicine or medical device in terms of the Medicines Act, further classification would be applied in accordance with the Act, regulations, and applicable guidelines as determined by SAHPRA. For example, a medicine could be classified into a category as provided in Regulation 9 of the General Regulations published in terms of the Medicines Act. A medical device would be further classified in accordance with SAHPGL-MD-04 Classification of Medical Devices and IVDs.

5.2 Request for classification of a borderline product

The process for classification of a borderline product may be initiated by a prospective applicant before the submission of an application for registration of a health product in terms of the Medicines Act. Alternatively, the process may be proposed to the applicant by SAHPRA after consideration of an application for

registration of a product. In either case, the applicable fee is payable by the applicant.

Recommendations as to whether a borderline product falls under the jurisdiction of the Medicines Act are made by a Technical Advisory Committee (TAC) appointed by the Chief Executive Officer of SAHPRA. The TAC will consist of representatives of relevant units within SAHPRA and external experts in the areas of foodstuffs, cosmetics, and disinfectants.

SAHPRA will provide the decision to the applicant as to whether the product is deemed to be a medicine, medical device or scheduled substance regulated under the Medicines Act. If it is deemed to fall outside the jurisdiction of SAHPRA, a recommendation may be made to the applicant regarding an appropriate alternative entity e.g. the Food Control Directorate of the National Department of Health for further input.

SAHPRA may change its view regarding the classification of the product in the event of any information or evidence that has a bearing on the status of the product. This may relate to a change in the legislation relating to the product (for example definitions), a change in scientific knowledge or even the way in which the product is packaged, promoted, or presented. SAHPRA can provide no assurance that any product, including products under development, will not subsequently be reclassified (for example, as a medicine or medical device). SAHPRA will engage with the holder of the certificate of registration, or the applicable licence holder regarding such a change in classification.

SAHPRA must also be informed if there is an intention to make a change to the claims made relating to a product or to the composition thereof.

5.3 Regulatory compliance

In cases where a product is being marketed which may be considered to be a health product but is not appropriately registered or authorised by SAHPRA, such a health product may also be referred to the TAC, referred to in 5.2 above in cases where there is uncertainty and/or a dispute concerning the classification thereof.

6. SUBMITTING AN APPLICATION

An application form must be submitted to SAHPRA. The completed submission must include the following documentation:

- (a) Table of contents (paginated with hyperlinks to sections);
- (b) Cover letter;
- (c) Labelling including the professional information, patient information leaflet, and/or label (where applicable);
- (d) Information for use, user manual, marketing material, or brochure (where applicable);
- (e) Safety data sheet (where applicable);
- (f) Report of the results on developmental work (where applicable);
- (g) Proposed classification with supporting rationale, including composition of the product, primary and secondary (where applicable) mode of action with supporting evidence (where applicable), and any proposed claims (explicit and/or implicit);
- (h) Risk assessment file (in accordance with ISO 14971 in the case of medical devices and ICH Q9 Quality Risk Management Guidelines in the case of medicines) and, where applicable, a risk management plan;
- (i) Classification of the product in other jurisdictions recognised by SAHPRA (where relevant);
- (j) Proof of payment.

The applicant must ensure that all relevant fields are completed, and all supporting documentation is attached. Incomplete applications will be identified as deficient, and review will not be progressed until deficiencies are addressed.

The prescribed fee and proof of payment must accompany the application. For the current fee payable, refer to the *Regulations regarding fees payable in terms of the provisions of the Medicines and Related Substances Act, 1965 (Act 101 Of 1965)* as published in the Government Gazette and on the SAHPRA website.

The completed application should be submitted electronically to: borderlineproducts@sahpra.org.za using the identified code.

6.1 Processing of applications

Applications for classification of borderline products will be processed by SAHPRA within 30 working days of receiving a complete and valid submission.

Upon receiving an application, SAHPRA will screen the application to ascertain whether all required documents as outlined above have been submitted. In cases where outstanding documents or other deficiencies are identified, SAHPRA will inform the applicant within 20 working days.

Applicants will be allowed two opportunities to respond to queries. If the applicant fails to submit outstanding documents and/or address deficiencies within a stated timeframe, the application will be considered incomplete and rejected. Any applicable fee already paid will be forfeited.

Once all required information has been received and evaluated, SAHPRA will issue a final response letter to the applicant confirming whether the product falls under the jurisdiction of SAHPRA or provide a recommendation as to the entity that could be approached for further advice. This letter will serve as SAHPRA's official position on the classification of the product.

Applicants who are aggrieved by the decision made by SAHPRA may appeal against such a decision in accordance with section 24A of the Medicines Act.²

If it is found that a product does fall under the jurisdiction of SAHPRA, a separate application for registration (for example, for a medicine) must be submitted as a separate process together with the applicable fee.

² Refer to the SAHPRA guideline SAHPGL-LEG-01 Guideline for appeals against Regulatory Decisions

7. REFERENCES

The following related documents are referenced:

7.1 Acts and Regulations

- 7.1.1 Medicines and Related Substances Act, 1965 (Act 101 of 1965) and regulations published in terms of that Act;
- 7.1.2 Foodstuffs, Cosmetics and Disinfectants Act, 1972 (Act 54 of 1972) and regulations published in terms of that Act;
- 7.1.3 The National Health Act, 2003 (Act 61 of 2003);
- 7.1.4 The Hazardous Substances Act, 1973 (Act 15 of 1973);
- 7.1.5 The National Regulator for Compulsory Specification Act, 2008 (Act 4 of 2008);
- 7.1.6 The Legal Metrology Act, 2014 (Act 9 of 2014);
- 7.1.7 WHO QAS/16.664, 2026.

7.2 Guidelines

- 7.2.1 SAHPGL-MD-04 Classification of Medical Devices and IVDs (2025)
- 7.2.2 SAHPGL-LEG-01 Guidelines for appeals against Regulatory decision (2024)

8. VALIDITY

This guideline is valid for a period of five years from the date it was approved. It will be reviewed when the validity period expires or as and when required.

ANNEXURE A

